

For the use only of a Registered Medical Practitioner or a Hospital or a Laboratory

Fincar

(Finasteride Tablets USP 5 mg)

Composition

Each film coated tablet contains:
Finasteride BP.5 mg

Description

Blue coloured circular, biconvex, film coated tablets, plain on both sides

Dosage Form

Tablets

Pharmacology

Pharmacodynamics

The development and enlargement of the prostate gland is dependent on the potent androgen, 5alpha-dihydrotestosterone (DHT). Type II 5alpha-reductase metabolizes testosterone to DHT in the prostate gland, liver and skin. DHT induces androgenic effects by binding to androgen receptors in the cell nuclei of these organs.

Finasteride is a competitive and specific inhibitor of Type II 5 alpha-reductase with which it slowly forms a stable enzyme complex. In man, a single 5-mg oral dose of finasteride produces a rapid reduction in serum DHT concentration, with the maximum effect observed 8 hours after the first dose. The suppression of DHT is maintained throughout the 24-hour dosing interval and with continued treatment. Daily dosing of finasteride at 5 mg/day for up to 4 years has been shown to reduce the serum DHT concentration by approximately 70%. The median circulating level of testosterone increased by approximately 10-20% but remained within the physiologic range.

In patients with BPH treated with finasteride (1-100 mg/day) for 7-10 days prior to prostatectomy, an approximate 80% lower DHT content was measured in prostatic tissue removed at surgery, compared to placebo; testosterone tissue concentration was increased up to 10 times over pretreatment levels, relative to placebo. Intraprostatic content of prostate-specific antigen (PSA) was also decreased.

In healthy male volunteers treated with finasteride for 14 days, discontinuation of therapy resulted in a return of DHT levels to pretreatment levels in approximately 2 weeks. In patients treated for three months, prostate volume, which declined by approximately 20%, returned to close to baseline value after approximately three months of discontinuation of therapy.

Pharmacokinetic:

Absorption

In a study of 15 healthy young subjects, the mean bioavailability of finasteride 5-mg tablets was 63% (range 34-108%), based on the ratio of area under the curve (AUC) relative to an intravenous (IV) reference dose. Maximum finasteride plasma concentration averaged 37 ng/mL (range, 27-49 ng/mL) and was reached 1-2 hours postdose. Bioavailability of finasteride was not affected by food.

Distribution

Mean steady-state volume of distribution was 76 liters (range, 44-96 liters). Approximately 90% of circulating finasteride is bound to plasma proteins. There is a slow accumulation phase for finasteride after multiple dosing. Finasteride has been shown to cross the blood brain barrier but does not appear to distribute preferentially to the CSF.

Metabolism

Finasteride is extensively metabolized in the liver, primarily via the cytochrome P450 3A4 enzyme subfamily. Two metabolites, the t-butyl side chain monohydroxylated and monocarboxylic acid metabolites, have been identified that possess no more than 20% of the 5(alpha)-reductase inhibitory activity of finasteride.

Excretion

In healthy young subjects (n=15), mean plasma clearance of finasteride was 165 mL/min (range, 70-279 mL/min) and mean elimination half-life in plasma was 6 hours (range, 3-16 hours). Following an oral dose of 14 C-finasteride in man (n=6), a mean of 39% (range, 32-46%) of the dose was excreted in the urine in the form of metabolites; 57% (range, 51-64%) was excreted in the feces.

Indications

FINCAR is indicated for the treatment and control of benign prostatic hyperplasia (BPH) in patients with an enlarged prostate to:

- cause regression of the enlarged prostate, improve urinary flow and improve the symptoms associated with BPH
- reduce the incidence of acute urinary retention and the need for surgery including transurethral resection of the prostate (TURP) and prostatectomy.

FINCAR administered in combination with the alpha-blocker doxazosin is indicated to reduce the risk of symptomatic progression of BPH

Dosage and Administration

The recommended adult dose is one 5 mg tablet daily, with or without food.

Although early improvement in symptoms may be seen, treatment for at least six months may be necessary to assess whether a beneficial response has been achieved. Thereafter, treatment should be continued long term.

No dosage adjustment is required in the elderly or in patients with varying degrees of renal insufficiency (creatinine clearances as low as 9 mL/min).

There are no data available in patients with hepatic insufficiency.

Fincar is contra-indicated in children.

Contraindications

FINCAR is not indicated for those with hypersensitivity to any component of this medication.

FINCAR is not indicated for use in women and children. Finasteride use is contraindicated in women when they are or may potentially be pregnant.

Warnings and Precautions

General

Prior to initiating therapy with finasteride, appropriate evaluation should be performed to identify other conditions such as infection, prostate cancer, stricture disease, hypotonic bladder or other neurogenic disorders that might mimic BPH.

Patients with large residual urinary volume and/or severely diminished urinary flow should be carefully monitored for obstructive uropathy. These patients may not be candidates for finasteride therapy.

Increased Risk of High-Grade Prostate Cancer

Men aged 55 and over with a normal digital rectal examination and PSA $\leq$ 3.0ng/ml at baseline taking finasteride 5mg/day in the 7-year Prostate Cancer Prevention Trial (PCPT) had an increased risk of Gleason score 8-10 prostate cancer (finasteride 1.8% vs placebo 1.1%). Similar results were observed in a 4-year placebo-controlled clinical trial with another 5-alpha reductase inhibitor (dutasteride, AVODART) (1% dutasteride vs 0.5% placebo). 5-alpha reductase inhibitors may increase the risk of development of high-grade prostate cancer. Whether the effect of 5-alpha reductase inhibitors to reduce prostate volume, or study-related factors, impacted the results of these studies has not been established.

Effect on PSA & prostate cancer detection

Finasteride causes a decrease in serum PSA levels by approximately 50% in patients with BPH, even in the presence of prostate cancer. This decrease is predictable over the entire range of PSA values, although it may vary in individual patients. Analysis

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PACKAGING DEVELOPMENT

Product Name: Fincar 5 mg		Material No.:	Version: 01	Item: Leaflet	Co-ordinator: Vinod	Artist: Vaibhav	Date: 30-9-19	
Colours: BLUE WOOL TEST VALUE 5-8 (LIGHT FASTENING DATA) Black					INK: Oil based Ink from DIC OR MICRO			
Design: Unfolded			Supersedes: 21079411		Software: Illustrator CC			
Fonts: -----				Links:				
Actual Size: 150 x 200 mm		Size after folding:	Pharmacode :		Grain Direction : Parallel to printing length		Screen : # __	
Material: 54 GSM Maplitho Paper					Varnish:		Artwork Print Size: ☐ actual ☐ scaled	
Path: PC: F:\Vaibhav\Vinod\xxxxxxx Fincar 5 mg Leaflet Malaysia.ai								
• Instructions / Remark: • Any deviation must be brought to the notice of packaging development co-ordinator immediately. • For any clarification, please contact packaging development co-ordinator immediately. NO CHANGES IN ARTWORK SHOULD BE DONE BY THE PRINTER • The printer should verify the e-proof against the approved artwork before submitting for approval and the e-proof should have printer details .				Checked by	Artist	Cordinator	file loaded in Server	Section Head
				Pharma Code	☐	☐		
				2D Code	☐	☐		
				QR Code	☐	☐		
				Bar Code	☐	☐		
				Artwork	☐	☐		
Spell check	☐	☐						

of PSA data from over 3000 patients in PLESS confirmed that in typical patients treated with finasteride for six months or more, PSA values should be doubled for comparison with normal ranges in untreated men. This adjustment preserves the sensitivity and specificity of the PSA assay and maintains its ability to detect prostate cancer.

Percent free PSA (free to total PSA ratio) is not significantly decreased by finasteride. The ratio of free to total PSA remains constant even under the influence of finasteride. If clinicians elect to use percent free PSA as an aid in the detection of prostate cancer in men undergoing finasteride therapy, no adjustment to its value appears necessary.

Increased risk of Breast cancer

Breast cancer has been reported in men taking finasteride 5 mg during the post-marketing period. Physicians should instruct their patients to promptly report any changes in their breast tissue such as lumps, pain, gynaecomastia or nipple discharge.

Information for patients

Physicians should inform patients that the volume of ejaculate may be decreased in some patients during treatment with **FINCAR**. This decrease does not appear to interfere with normal sexual function. However, impotence and decreased libido may occur in patients treated with **FINCAR**.

Drug/Laboratory test interactions

In patients with BPH, finasteride has no effect on circulating levels of cortisol, estradiol, prolactin, thyroid-stimulating hormone or thyroxine. No clinically meaningful effect was observed on the plasma lipid profile (i.e., total cholesterol, low density lipoproteins, high density lipoproteins and triglycerides) or bone mineral density. Increases of about 10% were observed in luteinizing hormone (LH) and follicle-stimulating hormone (FSH) in patients receiving finasteride, but levels remained within the normal range. In healthy volunteers, treatment with finasteride did not alter the response of LH and FSH to gonadotropin-releasing hormone indicating that the hypothalamic-pituitary-testicular axis was not affected.

Treatment with finasteride for 24 weeks to evaluate semen parameters in healthy male volunteers revealed no clinically meaningful effects on sperm concentration, mobility, morphology or pH. A 0.6 mL (22.1%) median decrease in ejaculate volume with a concomitant reduction in total sperm per ejaculate was observed. These parameters remained within the normal range and were reversible upon discontinuation of therapy with an average time to return to baseline of 84 weeks.

Interactions with other medicaments

No drug interaction of clinical importance with concomitant use of antipyrine, propranolol, digoxin, theophylline and warfarin has been identified.

Although specific interaction studies were not performed, finasteride was concomitantly used in clinical studies with acetaminophen, acetylsalicylic acid, (alpha)-blockers, angiotensin-converting enzyme (ACE) inhibitors, analgesics, anti-convulsants, beta-adrenergic blocking agents, diuretics, calcium channel blockers, cardiac nitrates, HMG-CoA reductase inhibitors, nonsteroidal anti-inflammatory drugs (NSAIDs), benzodiazepines, H₂ antagonists and quinolone anti-infectives without evidence of clinically significant adverse interactions.

Pregnancy and lactation

Pregnancy: Category X

FINCAR is not indicated for use in women

FINCAR is contra-indicated in women who are or may potentially be pregnant. Because of the ability of Type II 5 α-reductase inhibitors to inhibit conversion of testosterone to dihydrotestosterone, these drugs, including finasteride, may cause abnormalities of the external genitalia of a male foetus when administered to a pregnant woman. It is for these reasons that **FINCAR** is contra-indicated in women who are or may potentially be pregnant.

Exposure to finasteride - risk to male foetus

Women should not handle crushed or broken tablets of **FINCAR** when they are or may potentially be pregnant because of the possibility of absorption of finasteride and the subsequent potential risk to a male foetus.

Small amounts of finasteride have been recovered from the semen in subjects receiving finasteride 5 mg/day. It is not known whether a male foetus may be adversely affected if his mother is exposed to the semen of a patient being treated with finasteride. When the patient's sexual partner is or may potentially be pregnant, the patient is recommended to minimise exposure of his partner to semen.

Lactation: **FINCAR** is not indicated for use in women. It is not known whether finasteride is excreted in human milk.

Undesirable effect:

FINCAR is well tolerated, the adverse reactions usually have been mild and transient. The common adverse reactions are: Impotence, decreased libido, decreased volume of ejaculate, ejaculation disorder, breast enlargement, breast tenderness and rash. The incidence of new drug-related sexual adverse experiences decrease with duration of treatment.

In post marketing experience, following additional adverse experiences have been reported:

- Hypersensitivity reactions, including pruritus, urticaria and swelling of the lips and face
- Testicular pain.

In addition, male breast cancer has been reported in clinical trials and post marketing use.

Overdosage

Patients have received single doses of finasteride up to 400 mg and multiple doses of finasteride up to 80 mg/day for three months without adverse effects. Until further experience is obtained, no specific treatment for an overdose with finasteride can be recommended.

Shelf life

36 months

Storage

Store below 30°C.

Pack size

Carton containing 10 blisters of 10 tablets each

Registration holder in Malaysia

CIPLA MALAYSIA SDN BHD
Suite 1101, Amcorp Tower, Amcorp Trade Centre,
18 Persiaran Barat, 46050 Petaling Jaya,
Selangor, Malaysia.

Manufactured by

Cipla Ltd.
S-103 to S-105, S-107 to S-112,
L-147 to L-147-1, Verna Industrial Estate, Verna Goa, India.

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